

Merative admission grouping: nine-step comparison

Legacy: Advantage Suite v5.4, Data Manager’s Guide, Chapter 9. **New:** Flexible Analytics Inpatient Admission Grouper Methodology Guide v11.2.0.

How to read this: This is a conceptual sequence, not a verified internal execution order. First identify room-and-board anchors, then order them for window evaluation. Transfer checks happen during window construction. Calculate the preliminary totals needed for deconstruction before applying the removal rule. **Stages 1–6 provisionally group claims; stages 7–9 calculate fields, may remove candidate admissions, and publish results.** Differences in documentation do not alone prove differences in a production claims population.

1–3 • Select and prepare claims

Stage	Legacy v5.4	New v11.2.0	What the documents support
1. Select eligible input	Independent Admission Build batch process after the database has been built. In Rebuild, candidate facility headers/services and professional services need valid Date of First Service and medical coverage under Claim Type Code Medstat, plus either inpatient acute/non-acute/missing Place Group or qualifying outpatient ER/Observation status (facility flag or professional Place of Service evaluation). Update also requires new qualifying patient activity since the prior build. LEGACY pp. 9-1–9-2.	One required input: the medical-claims file; admissions are built one patient at a time. Facility and professional medical claims with <code>claim_type_cd=1</code> are eligible to join , not guaranteed to join. Inpatient place groups are 1–3; only inpatient facility room-and-board claims construct windows. NEW pp. 26, 32.	Unknown scope comparison: no supplied field crosswalk establishes a wider v11 intake. These pages do not establish API call count, per-patient full rebuild, or absence of incremental processing.
2. Identify room-and-board anchors	Facility headers containing a room-and-board service line, identified by Room and Board Flag Code, provide the window basis; how that flag is populated is not specified here. LEGACY pp. 9-1, 9-5.	For inpatient facility records, a valid <code>rb_indicator</code> identifies room-and-board records. If omitted from the header, it is derived using <code>rev_code</code> and <code>claim_id</code> . Let A= 0118/0128/0138/0148 ; B= 0100-0169 or 0180-0219 , excluding A; C= 0170-0179 ; D= 1000-1005 ; E=other. The combination matrix assigns 1 for B or D without C (A/E optional); 2 for B and C (A/D/E optional); 3 for C without A/B (D/E optional); 4 for A without B/C/D (E optional); otherwise blank. NEW pp. 27–28, 32.	Unknown cross-version anchor change: v11 derivation is conditional on a missing header indicator; legacy flag provenance and a field-level crosswalk are not documented. The v11 values are not single-revenue-code rules.
3. Order anchors	Room-and-board facility headers are ordered by Begin Date, then Calculated Days: for equal dates a one-day transfer-from claim precedes a longer transfer-to claim. Conditional priorities put died status later for same service date/days; adjustments/voids after originals; missing values and bulk adjustments last. Same-Person-ID combined mother/baby records favor the mother’s room-and-board flag; Medstat Record ID is the final nonmissing tie-breaker. LEGACY pp. 9-5–9-6.	The cited pages describe examining each <code>rb_indicator</code> record and comparing it with existing windows. A later field rule references the “sort order of the time window” but does not specify corresponding keys or adjustment/void precedence. NEW pp. 28–29, 36.	Unknown parity: do not infer the v11 sort keys, or infer that a rule absent from these pages is absent across v11.

4–6 · Form stays and associate claims

Stage	Legacy v5.4	New v11.2.0	What the documents support
4. Build/extend windows	Room-and-board record Begin Date is the later of Date of First Service and available claim-level Admission Date Original. Documented outcomes include fitting strictly inside the discharge boundary, right extension/end-on-discharge, next-day adjacency, or a new admission; a left extension of an already-established admission is documented for Rebuild. Transfer logic within the right-extension branch is conditional on Begin Date equaling discharge. LEGACY pp. 9-5–9-7.	When there is no patient window, the evaluated valid <code>rb_indicator</code> record creates one using its Begin/End Dates; the guide also names First/Last Service as an alternative date basis without stating precedence. Against existing windows, End Date on discharge fits; End Date after discharge can extend; next-day Begin Date is adjacent. If the record neither overlaps nor is adjacent , it starts a new window. NEW pp. 28–30.	Generally aligned structure, qualified differences: end-on-discharge appears in different branches, but no different final admission follows automatically. No separate backward-extension rule is stated on the cited v11 pages; this does not prove that v11 lacks the behavior.
5. Decide continuation/transfer during step 4	For specified right-extension/discharge-boundary and adjacency cases, Standard Transfer Logic checks rehab/non-rehab change; unequal valid Facility ID Standard; then, if IDs cannot be used, qualifying unequal discharge statuses when admission status indicates transfer or a Transfer From Type Code identifies a prior transfer-from admission; finally, unequal nonmissing Place Group Medstat when earlier checks are inconclusive for missing values. LEGACY pp. 9-7–9-8.	In a forward extension, transfer logic runs when record Begin Date equals discharge; it also runs for adjacency. A transfer creates another admission, otherwise the record can join the window. Tests include rehab/non-rehab <code>rb_indicator</code> change; unequal nonmissing <code>facility_id_std</code> ; status comparison when an ID is blank and admission discharge status indicates transfer; then unequal nonmissing place groups if either status is missing. Page 31 notes place-group augmentation using <code>std_place</code> , not a transfer field named <code>std_place_group</code> . NEW pp. 29–31.	Partly aligned, not “Same”: valid versus nonmissing IDs, the legacy Transfer From Type Code alternative, and fallback conditions are not established as operationally equivalent. No claim-level transfer result is proven.
6. Associate other claims/resolve two windows	Remaining facility claims and professional services generally join when service dates fall within the admission window, including its endpoints; an ER/Observation first-service date one day before also qualifies. For applicable two-window cases, a record with valid provider or nonmissing Place Group goes to a hold array; without either it goes to the first potential window. Hold resolution tries record-type-specific provider/facility matching, then nonmissing Place Group, then shorter stay, with first admission on an equal-duration tie. LEGACY pp. 9-12–9-14.	<code>claim_type_cd=1</code> facility/professional claims are eligible. The listed ER/Observation day-before/during qualification signals differ by record type: for facility claims, revenue 0450–0459, 0768, 0680–0689, 0981, standard POS 23, and procedure group 111; for professional claims, POS 23 or procedure group 111 with specified provider-code ranges or MDC ^{19/20} . Qualifying telehealth can join within a window; LTC is included by default, optionally excluded. For two windows, p. 32 prioritizes first matching <code>facility_id_std</code> (or <code>prov_id</code> if missing), otherwise shortest; p. 33 separately says earliest match on <code>facility_id_std</code> , <code>prov_id</code> , or <code>std_place_group</code> , otherwise shortest. NEW pp. 32–34.	Documented routing differences, unresolved v11 precedence: the two v11 descriptions are not reconciled here. Legacy-page silence on telehealth/LTC or ER identification does not prove those rules were absent; neither an expanded population nor a particular reassignment has been demonstrated.

7–9 · Calculate, filter, and publish

Stage	Legacy v5.4	New v11.2.0	What the documents support
7. Calculate attributes and preliminary aggregates	Configurable generic/B_ADM_RULES assignment supports First/Last Non-missing, Sum, Highest Sum and ranking; examples include net-payment sum and RVU-based primary physician. LOS starts with nonnegative room-and-board Days and is adjusted as records arrive. Admit Type/Source Code UB come from the first room-and-board window record if available. In a combined same-Person-ID mother/baby admission, mother's flag , hence age and sex, prevail—not her entire claim. Clinical assignments and Disease Staging are described as acute-only, but the DRG Grouper is also stated to be called for all admissions ; conditional PDX rotation and specified maternity/newborn corrections are documented. LEGACY pp. 9-6, 9-9–9-11, 9-14–9-21.	The cited field rules select latest nonmissing inpatient room-and-board facility records for <code>admit_source</code> , <code>admit_type</code> , age, sex and discharge status; sum facility financial amounts; and select the eligible primary physician by highest total RVUs (ASCII tie-break). LOS is discharge minus admit date, with same-day LOS = 1. <code>calculated_drg</code> , <code>drg_final</code> , <code>dxcat</code> and <code>stage</code> are defined; the DRG release defaults from discharge-year or explicit <code>drgVersion</code> . Date-pair precedence in the window rules is unstated. NEW pp. 28–30, 35–42.	Changed documented LOS formula and named-field sourcing , subject to configured legacy rules and field crosswalk; values need not differ. Do not infer v11 nonconfigurability, absence of mother/baby precedence, or a directly comparable DRG-release change. Legacy's clinical-scope statements must be reported in tension, not flattened into an acute-only DRG rule.
8. Deconstruct and finalize	Default deconstruction criterion is Total Allowed Amount (<code>adm_chg_allow_amt</code>) ≤ 0 , but client-specific <code>B_ADM_DECONST</code> criteria can replace it (AND within row, OR across rows). Admissions with a capitated room-and-board facility service or newborn age in months = 0 are protected. Update-mode Reconstruction is described for a record fitting an old window and the specified transfer-from-old-admission case; Recalculation reassigns all admission-level fields when a new non-room-and-board facility-header record affects an existing admission. LEGACY pp. 9-7, 9-14, 9-17–9-18.	An admission with <code>adm_cap_svc_ind="y"</code> is not deconstructed even when aggregates would otherwise qualify. The cited page does not state the aggregate threshold or Reconstruction/Recalculation/full-build semantics. NEW p. 38.	Unclear cross-version removal and incremental behavior : both document capitation-related protection but not proven equivalent trigger populations. Preliminary totals necessarily precede their threshold test conceptually; the precise internal sequencing/finalization remains unverified.
9. Publish outputs/IDs	Standard admission database tables include <code>F_ADMIT</code> , <code>D_ADMIT_DX</code> , <code>D_ADMIT_OTHER</code> , <code>D_GRPR_DX</code> , <code>D_GRPR_ANY_DX</code> , <code>D_GRPR_PROC</code> , <code>D_GRPR_ANY_PROC</code> , <code>D_STD_HOSP</code> , and claim link <code>A_ADMIT_CLM</code> . Unique Admit ID concatenates Patient Key, Admit Start Period Key and a two-digit sequence; on update/rebuild it persists unless admission dates change. LEGACY pp. 9-3–9-4, 9-9.	Named files include Admissions, Admissions-medical link, ungrouped records, and admission-episode link for a same-run episode grouper; <code>statistics.csv</code> is conditional on the statistics API parameter being TRUE. <code>admission_id</code> is a patient sequence starting at one followed by a seven-digit number calculated from admission date. The cited ID page does not specify rerun stability. NEW pp. 36, 44–45.	Changed documented interfaces and ID constructions , not a proven one-to-one schema migration, universal ID mismatch, or v11 instability.

Open questions for Merative: v11 input-to-legacy field mapping; v11 window sort/date precedence and multi-window tie-breaks; supported incremental/rebuild behavior; complete v11 deconstruction threshold and exemptions; and a same-claims comparison before asserting actual split/merge or population changes.